

MEDICAL HISTORY SUMMARY

Medical Examiner Case Investigation

Case Number:	ME-2024-001-ALK
Decedent:	CONFIDENTIAL
Date of Birth:	March 15, 1982
Age at Death:	42 years
Date of Death:	November 13, 2024 (estimated)
Compiled By:	Medical Examiner Investigator J. Patterson
Date Compiled:	November 17, 2024

SOURCES OF INFORMATION

This medical history summary is compiled from multiple sources including: medical records from Riverside Medical Group (primary care physician Dr. James Reynolds, MD), emergency department records from Metro General Hospital, pharmacy records, family member interviews, and employer health records. Records were obtained through proper legal authorization in accordance with state and federal regulations.

PRIMARY CARE PHYSICIAN RECORDS

Established Care and Recent Visits

The decedent established care with Dr. James Reynolds at Riverside Medical Group in June 2016. Medical records document approximately 12 office visits over the eight-year period, with decreasing frequency in recent years. The most recent documented visit was March 2023, approximately 20 months prior to death.

Documented Medical Conditions

Alcohol Use Disorder: First formally documented in medical records in August 2018, though retrospective review suggests problematic alcohol use may have been present earlier. Records indicate the patient reported daily alcohol consumption with intake estimated at 8-12 standard drinks per day. The patient acknowledged difficulty cutting down or controlling alcohol intake. He reported

experiencing withdrawal symptoms (tremors, anxiety, sweating) when attempting to reduce consumption. The CAGE questionnaire administered in 2019 was positive for all four criteria. Dr. Reynolds documented multiple counseling sessions regarding alcohol use and offered referrals to addiction specialists and treatment programs. Records indicate the patient declined formal addiction treatment on multiple occasions.

Gastroesophageal Reflux Disease (GERD): Diagnosed in 2017. Treated with over-the-counter antacids. Symptoms were noted to worsen with alcohol consumption.

Mild Hypertension: Documented in 2020 with blood pressure readings typically in the 135-145/85-92 range. Patient was counseled regarding lifestyle modifications including alcohol reduction, exercise, and diet. No antihypertensive medications were prescribed as patient expressed preference to try lifestyle modifications first. Subsequent follow-up was inconsistent.

Obesity: Body Mass Index (BMI) ranged from 27.8 to 29.4 across documented visits, classifying the patient as overweight to obese. Patient was counseled regarding weight management.

Laboratory Findings

Laboratory work from March 2023 visit (most recent): Aspartate aminotransferase (AST): 89 U/L (normal 10-40), Alanine aminotransferase (ALT): 56 U/L (normal 7-56), AST/ALT ratio: 1.6, Gamma-glutamyl transferase (GGT): 127 U/L (normal 9-48), Alkaline phosphatase: 98 U/L (normal 44-147), Total bilirubin: 1.3 mg/dL (normal 0.1-1.2). These findings were consistent with alcohol-induced hepatic dysfunction. Complete Blood Count showed macrocytic anemia with MCV of 104 fL (normal 80-95), consistent with chronic alcohol use. Platelet count was at the lower end of normal at 165,000 (normal 150,000-400,000). Lipid panel showed elevated triglycerides at 198 mg/dL. Fasting glucose was normal at 94 mg/dL.

Previous laboratory work from 2021 showed similar but less pronounced abnormalities, indicating progressive hepatic dysfunction over time. Dr. Reynolds documented counseling regarding these findings and strongly recommended alcohol cessation with specialist referral.

Medications and Treatment

The patient was prescribed the following medications at various times, though adherence was questionable: Omeprazole 20mg daily for GERD, Thiamine (Vitamin B1) 100mg daily (for alcohol-related deficiency), Folic acid 1mg daily (for anemia). No medications for alcohol use disorder (such as naltrexone, acamprosate, or disulfiram) were prescribed as the patient declined these options. Pharmacy records indicate irregular prescription refills, suggesting poor medication compliance.

Missed Appointments and Non-Compliance

Medical records document multiple no-shows for scheduled appointments. Dr. Reynolds' notes express concern about the patient's inconsistent follow-up care and failure to address alcohol use disorder adequately. Documentation indicates the patient minimized his alcohol consumption during visits and was resistant to formal addiction treatment despite strong medical recommendations.

EMERGENCY DEPARTMENT RECORDS

Visit #1: August 14, 2022

The patient presented to Metro General Hospital Emergency Department at 03:15 hours via ambulance after being found intoxicated and disoriented by police during a welfare check. Chief complaint documented as 'intoxication.' Vital signs on arrival: BP 148/94, HR 102, RR 18, Temp 98.4°F, SpO2 96% on room air. Physical examination revealed an obviously intoxicated male with slurred speech, ataxic gait, and decreased level of alertness. Blood alcohol concentration was 0.32% (320 mg/dL). Comprehensive metabolic panel showed mildly elevated liver enzymes. The patient was observed in the emergency department for several hours until showing clinical improvement. Social work consultation was provided. Patient was given resources for addiction treatment and strongly encouraged to follow up with primary care and addiction services. Patient left AMA (against medical advice) before complete sobering.

Visit #2: February 3, 2023

Presented via EMS after neighbor called 911 for welfare check. Found heavily intoxicated at home. Chief complaint: 'Found down, intoxicated.' Vital signs: BP 152/89, HR 96, RR 16, Temp 98.1°F. Physical exam notable for profound intoxication with decreased responsiveness, requiring stimulation to maintain wakefulness. BAC 0.28% (280 mg/dL). Labs showed elevated liver enzymes (AST 102, ALT 67). Patient admitted to regular daily drinking. Psychiatric evaluation determined no acute suicidal ideation or psychiatric emergency. Patient offered admission for detoxification, which he declined. Discharged home with referrals to outpatient addiction services. Patient did not follow up with recommended services.

Visit #3: July 19, 2023

Self-presented to emergency department at 11:30 hours complaining of abdominal pain and nausea. Patient reported recent heavy alcohol consumption. Examination revealed epigastric tenderness. Labs showed lipase elevation (245 U/L, normal <160), concerning for pancreatitis. Liver enzymes significantly elevated (AST 134, ALT 89). BAC 0.18% (180 mg/dL) despite being mid-day. Diagnosis of acute alcoholic pancreatitis. Patient was admitted to hospital for management. Hospital course was unremarkable with conservative management. During admission, addiction medicine consultation was requested. Patient engaged initially but ultimately declined formal treatment program enrollment. Discharged after 3 days with instructions for abstinence from alcohol, primary care follow-up, and addiction treatment referrals. Records indicate patient did not attend scheduled follow-up appointments.

SOCIAL HISTORY AND FAMILY INTERVIEWS

Employment History

The decedent was employed as a software developer at TechCorp Solutions for eight years prior to death. According to employer representatives and HR records, his work performance had been satisfactory until approximately two years ago when colleagues noted increasing absences and decreased productivity. The decedent had exhausted his sick leave and was placed on performance

improvement plan in early 2023 but showed improvement sufficient to remain employed. Employer was not aware of specific substance abuse issues but had referred employee to Employee Assistance Program (EAP) resources, which records indicate he did not utilize.

Family History

Information obtained from the decedent's sister (next of kin): The decedent was divorced in 2019 after seven years of marriage. Ex-spouse cited alcohol use as a contributing factor to marital dissolution. The decedent had no children. Family history is significant for alcohol use disorder affecting the decedent's father and paternal uncle, suggesting potential genetic predisposition. The decedent's mother died of cancer in 2015. The decedent became increasingly isolated from family members over the past 3-4 years, with decreasing frequency of contact.

Substance Use History

According to family and medical records, the decedent began drinking alcohol in college but consumption escalated significantly following his divorce in 2019. Family reported that they had expressed concerns about his drinking on multiple occasions and encouraged him to seek treatment. The decedent reportedly acknowledged his drinking was problematic but repeatedly stated he would 'cut back on his own' and did not follow through with treatment recommendations. Family was not aware of illicit drug use. Medical records and toxicology from previous ED visits show no evidence of other substance use.

Mental Health History

The decedent never engaged in formal mental health treatment, and no psychiatric diagnoses are documented in available medical records. However, family members reported that he seemed increasingly depressed following his divorce, became more socially isolated, and appeared to use alcohol as a coping mechanism. During emergency department visits, psychiatric evaluations found no acute suicidal ideation or intent to self-harm. The decedent denied any history of suicide attempts. Depression screening tools administered during medical visits suggested mild to moderate depressive symptoms, but patient declined mental health referrals.

Social Isolation

Multiple sources including family, neighbors, and coworkers reported that the decedent had become progressively more isolated over the past 2-3 years. He declined social invitations, reduced contact with family members, and spent increasing amounts of time alone in his apartment. This social withdrawal appeared to correlate with increasing alcohol consumption.

SUMMARY AND ANALYSIS

The decedent had a well-documented history of alcohol use disorder spanning at least 6-7 years, with progressive escalation of consumption and development of physiological dependence. Multiple healthcare encounters documented the serious health consequences of chronic alcohol abuse, including alcoholic hepatopathy with progressive liver dysfunction, pancreatitis, and multiple episodes of severe acute intoxication requiring emergency care.

Despite repeated medical counseling, referrals to addiction treatment services, and family interventions, the decedent did not engage in formal substance abuse treatment. This pattern of declining treatment, inconsistent medical follow-up, and progressive social isolation is unfortunately common in individuals with severe alcohol use disorder and represents significant risk factors for alcohol-related mortality.

The constellation of documented medical findings including elevated liver enzymes, macrocytic anemia, prior episodes of acute intoxication, and evidence of alcohol-induced organ damage are entirely consistent with chronic, heavy alcohol use. The medical history strongly supports the autopsy findings and provides context for understanding the progression to fatal alcohol intoxication.

Compiled by: J. Patterson, Medical Examiner Investigator

Date: November 17, 2024